

A unified repertoire of indel mutational signatures in 6,975 tumors

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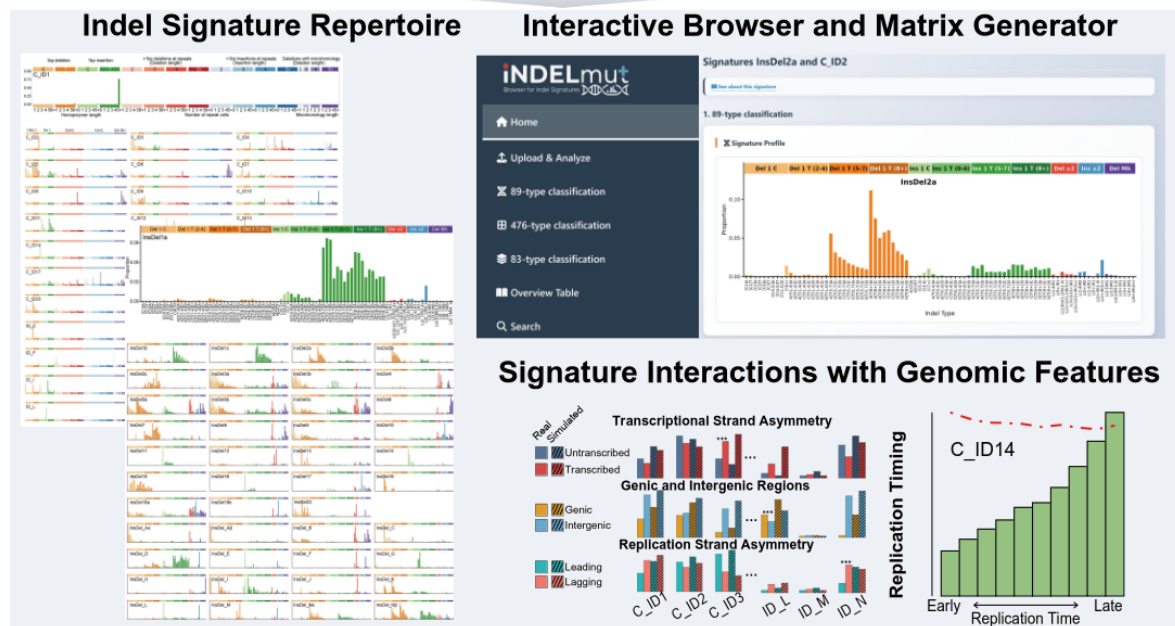
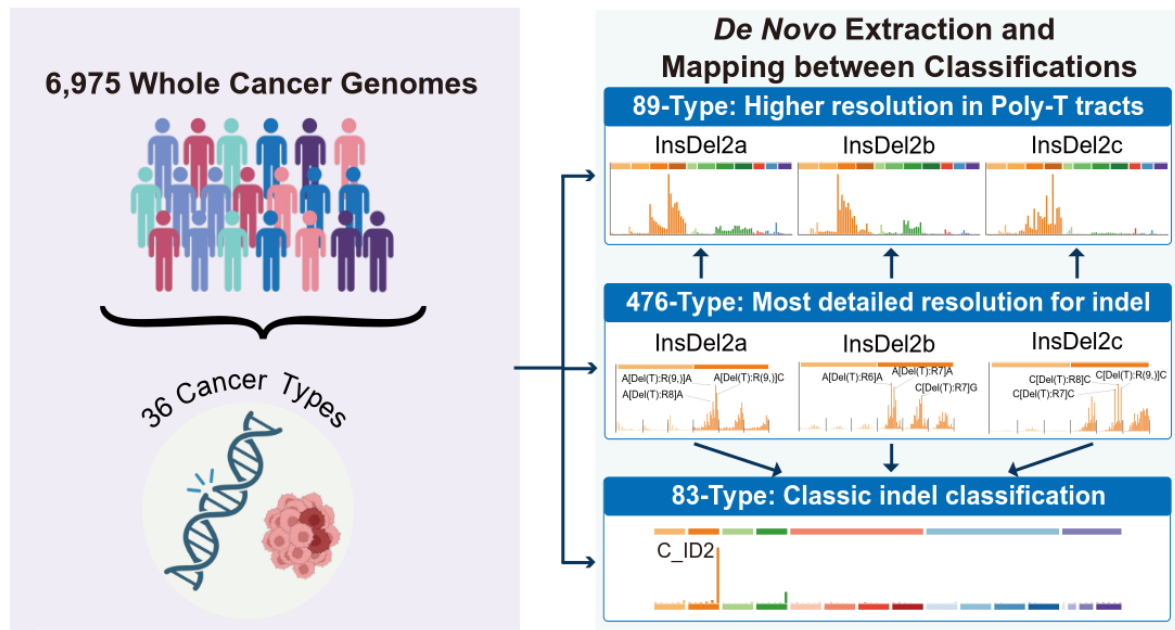
Summary

We present a unified repertoire of indel mutational signatures in 6,975 tumors across 36 cancer types. Small insertions and deletions (“indels”) are an important under-studied class of mutations. Koh, Nanda, and colleagues recently introduced two richer indel classifications, with 89 and 476 indel types. We identified signatures separately in the two new classification schemes and the widely used 83-type classification scheme using two algorithms. We identified 44 89-type signatures and their 476-type counterparts; 26 are new. Because lossless translation between the 83-type and newer schemes is impossible, we used tumors dominated by single signatures to exhaustively map correspondences across classifications. The 89- and, especially, the 476-type schemes distinguish signatures and spectra that the 83-type scheme cannot. We release software for generating and plotting spectra in the new classifications. An accompanying web site enables exploration of signatures and their cross-classification relationships and provides a service to construct spectra from VCF files.

Graphical Abstract

Highlights

1. Repertoire of indel signatures across 6,975 genomes in 3 indel classification schemes
2. Identified 26 novel 89-type and 11 novel 83-type classification indel signatures
3. Integrative topography across transcription, replication, and genic regions
4. Website and software for signature browsing and matrix generation from VCF files



Introduction

Somatic mutations are caused by various mutational processes and represent a driving force behind tumorigenesis and cancer development (Alexandrov et al. 2014). These mutations can result from either endogenous sources, such as 5-methylcytosine deamination or defective DNA repair mechanisms (Davies et al. 2017; Cooper et al. 2010; Grolleman et al. 2019; Boot et al. 2022), or exogenous sources, including exposure to chemical carcinogens in tobacco smoke or certain herbal medicines (Alexandrov et al. 2016; Ng et al. 2017; Dziubańska-Kusibab et al. 2020; Boot et al. 2020). Mutational signature analysis provides insights into cancer etiology, prognosis, prevention, evolution, and mutational signatures can also serve as biomarkers for mutagenic exposures

By “mutational signatures” we mean distinctive patterns of mutations left on genomes by mutagenic processes or exposures. They can be identified by exposing cultured cells, organoids, or experimental animals to suspected mutagens or perturbing DNA repair pathways and then sequencing the affected genomes (Boot et al. 2018; Huang et al. 2017; Kucab et al. 2019; Caipa Garcia et al. 2024; Riva et al. 2020). In addition, machine learning can identify mutational signatures as latent factors that explain the patterns of mutations in large collections of somatic mutation data (Alexandrov et al. 2020; Alexandrov et al. 2014; Nik-Zainal et al. 2012; Chen et al. 2024; Koh et al. 2025; Degasperi et al. 2022; Jin et al. 2024). For example, data mining of upper tract urothelial cancers (UTUC) from Taiwan initially identified the aristolochic acid (AA) single-base substitution (SBS) signature (Hoang et al. 2013; Poon et al. 2013). Subsequent analysis revealed that this signature was also present in bladder, kidney, and liver cancers (Ng et al. 2017; Poon et al. 2015). More recently, data mining of Chinese liver cancer genomes and experiments in cell culture showed that AA also generates double-base-substitution (DBS) signatures and, relevant to the current study, small insertion-and-deletion (indel) signatures (Figure 1) (Chen et al. 2024).

Mutational signature research has tended to focus less on indel rather than SBS signatures, and symptomatic of this COSMIC v3.6 catalogs only 25 indel signatures compared to 78 SBS signatures. Nevertheless, indel signatures can yield important information for cancer classification, epidemiology, and treatment and for understanding DNA repair and mutagenic mechanisms. Examples include identification of tumors with homologous recombination deficiency or with mismatch repair deficiency (Huang et al. 2015; Koh et al. 2025; Davies et al. 2017) and a rare indel hypermutator phenotype due to a mutation in topoisomerase 2 alpha (Boot et al. 2022).

The first widely used classification of indels recognized 83 indel types, (Alexandrov et al. 2020), and recently Koh et al. (2025) proposed two classification schemes, one with 89 types of indel and one with 476 types (Figure 1).

The older, 83-type scheme primarily categorizes indels based on the number of bases inserted or deleted, and for single-base insertions and deletions the identity of the deleted or inserted base (conventionally shown as pyrimidines, C or T). For these single-base indels, the classification also considers the number of flanking C or T residues (Figure 1A). Indels longer than 1 base are further classified by their occurrence within repetitive sequences (i.e. microsatellites, simple tandem repeats). For deletions of 2 bases the 83-type system classification also encodes the presence of microhomology. Microhomology is a hallmark of error-prone DNA repair by non-homologous end-joining, which operates when repair by homologous recombination is not available, notably in BRCA-deficient tumors. For example, the 3-base deletion (ACA|TCA|GG→ACAGG) exhibits a 2-base microhomology (CA).

One of the new classification schemes is based on 476 types of indel and one is based on 89 types, which mainly consolidates similar mutation types in the 476-type classification into coarser-grained mutation types (Koh et al. 2025) (Figure 1, Table S1). A prominent distinction is that, unlike the 83-type classification scheme, both the new classifications distinguish different types indels of a single T or C according to the non-T (respectively, non-C) flanking bases. This dramatically improves discrimination between mutational spectra and signatures containing single-base indels, which are present in most signatures.

For example, the 83-type classification distinguishes deletions of single Ts only by the number of T's at the site of the deletion: 1, 2, 3, 4, 5, or 6 Ts (Figure 1A). By contrast, the 89-type classification distinguishes among deletions of T according to bases preceding or following the sequence of Ts at the site of the deletion. However, it groups together deletions of T from sequences of 1, 2, 3, or 4 T, and groups together deletions of sequences of 5, 6, 7, 8 and Ts. Thus, for example, deletions of ATA→AA or ATTA→ATA are distinguished from deletions of CTA→CA or CTTA→CTA, but the deletions CTA→CA and CTTTTA→CTTTA have the same classification. The 476-type classification scheme provides fined-grained distinctions among deletions of a single C or T that combine the distinctions in the 83 and 89-type system. The 476-type classification scheme shows, for example, that in the signature in Figure 1 (InsDel23), the most prominent deletions are ATA→AA, CTA→CA, and GTA→GA. The classifications of indels of 2 or more bases are less intuitive. Table S1 provides a mapping from the 476-type classification to the 89 and 83-type classifications, as well as multiple examples of each category of deletion.

Here, we analyzed somatic indels in 6,975 whole genomes to create a comprehensive collection of indel signatures in all three classification schemes using approaches based on hierarchical Dirichlet processes and on non-negative matrix factorization. We systematically matched the 476-type signatures to 89-type signatures by collapsing the 476-type signatures to 89-type signatures and comparing the collapsed signatures to signatures identified in the 89-type classification. We used tumor samples with spectra dominated by single signatures to map signatures in the 83-type classification to signatures in the other two classifications. We further profiled signatures' replication timing, asymmetry between genic and intergenic regions and asymmetry between leading and lagging replication strands, providing insights into mutational processes underlying the signatures. We also tallied their associations to SBSs and to sex within individual cancer types. Finally, in addition to the supplementary information provided here, we also provide a web site to facilitate exploration of these signatures (<https://indelsig.net>)

Results

Repertoire of indel signatures based on de novo signature discovery

We identified indel signatures in all three classifications from a total of 6,975 cancer genomes encompassing 36 cancer types using a method based on hierarchical Dirichlet processes (Liu et al. 2023), and a method based on non-negative matrix factorization (Islam et al. 2022). The extracted signatures were broadly consistent between the two methods (Table 1, Tables S2-S8). We identified 44 mutational signatures in the new 89-type classification and, independently, the 44 corresponding signatures in the new 476-type classification. We assessed concordance between signatures identified in two classification systems by collapsing the 476-type signatures to their 89-type counterparts and comparing them to signatures extracted in the 89-type classification. Mean cosine similarity was

> 0.989 (median > 0.994 , minimum > 0.951), providing evidence that these signatures constitute latent factors robust to the feature set in which they were discovered. Further evidence of their robustness derives from the identification of tumor spectra dominated by a single signature. For 35 of the 44 signatures, there were tumor spectra with cosine similarity > 0.95 cosine similarity, and for 39 there were tumor spectra with similarity > 0.90 (Table 1). Of the 44 signatures identified in the 89-type classification, similar signatures were previously described for 18 (Koh et al. 2025) and 26 were not previously described to our knowledge (Table 1). We also carried out signature discovery in spectra in the 83-type classification, and discovered 34 signatures, of which 20 were similar to signatures in the COSMIC reference set, and 3 additional signatures were similar to signatures identified in Jin et al. (2024) (Table 1).

Because lossless translation between signatures in the 83-type and the two new classification schemes is impossible (Table S1), we used tumor mutational spectra dominated by individual signatures to systematically elucidate signature correspondences across classification schemes. Table 1 shows the correspondences, with details and discussion in <https://indelsig.net/>.

In several cases, the new indel classifications systems were able to distinguish signatures that the older system merged. This is clearest for deletions of single T bases from tracts of 6 Ts. Our data set had > 8.5 million of these deletions out of 18.4 million deletions in total. The COSMIC reference signature ID2 in the older, 83-type classification, consists exclusively of these deletions. The new indel classifications divide ID2 into three signatures. Among these, InsDel2b is dominated by deletions of T from tracts of 5 to 7 Ts. InsDel2a and InsDel2c are both dominated by deletions of single Ts' from stretches of 8 Ts. However, they are distinguished by InsDel2a's preference for flanking A bases and InsDel1c's preference for flanking C bases. Mutations attributed to Insdel2b and InsDel2c are strongly enriched among MSI-H tumors. Furthermore, the counts of mutations attributed to these signatures are correlated with the counts of mutations attributed to the MSI-H-associated SBS signatures SBS6, SBS26, and SBS44 (Figure 4, Figure S1).

In addition to the deletions of single T bases from long poly-T tracts, insertions of single T bases into long poly-T tracts also showed that the new indel classification systems distinguish signatures that the older system merged. Our data set had 3.3 million insertions of single T bases into tracts of 5 Ts, which in the 83-type system constitutes a single mutation class, and COSMIC reference signature ID1 consists exclusively of this class of insertions. The new indel classifications divide ID1 into three signatures. Among these, InsDella has the largest proportion of insertions into tracts of 8 Ts and InsDel1b has the smallest proportion, while InsDel1c has an intermediate proportion. Compared to InsDel1b, InsDella and InsDel1c have larger proportions of A to the left of the poly-T tract and G to the right, and smaller proportions of C to the left of the poly-T tract with C or A to the right. InsDel1b is further distinguished from InsDella and 1b in that several tumors with high proportions of InsDel1b (> 0.4) have mutations in the proofreading domain of polymerase epsilon (Table S9).

Two signatures in which the 89-type classification misses clear differences

While the 89-type classification system on the whole better discriminates signatures, in two cases the 83-type and 476 type systems clearly differentiated signatures that were almost identical to the 89-type system. One case consisted of InsDel13 and InsDel_C. These are nearly identical in the 89-type system, which groups deletions of single Ts in stretches of 1, 2, 3, or 4 Ts as a single deletion type. However the 83 and 476 type systems show that InsDel_C has deletions of 1 T from ATA (ATA $\rightarrow$ AA), while InsDel13 has deletions of T from ATTA (ATTA $\rightarrow$ ATA, Figure S2A). The other case consisted of InsDel11 and InsDel16. InsDel11 has insertions of one C next to an isolated

C (C with non-Cs on each side, e.g. ACG→ACCG) and insertion of a one T next to an isolated T (e.g. GTG→GTTG). InsDel16 has insertion of one C between non-Cs (e.g. AT→ACT), and insertion of one T between non-Ts (e.g. AG→ATG, Figure S2B). Again, the 89-type system does not capture these differences, while the 83 and 476-type systems do.

Clarified signatures of aristolochic-acid-induced deletions

InsDel23 is caused by aristolochic acid (AA) exposure and the 476-type signatures shows that most AA deletions involve 5'TA3'/3'AT5' → A/T (Figure 1, and see <https://indelsig.net>). This pattern is not visible in either the 83-type or 89-type classification XXXXXX Do the detailed spectrum of the exemplar XXXX One might speculate that this pattern is related to the adenine adducts caused by AA, which are also thought to be responsible for the predominance of A→T single base substitutions in the AA SBS signature and for the frequent involvement of adenines in the AA DBS signature.

APOBEC3A activity and InsDel_A

We identified InsDel_A , an indel signature that in the 476-type classification consists primarily of 1 bp C deletions TCA→TA and TCT→TT deletions (Figure S2C, right sub-panel). Thus, InsDel_A targets the same sequences as the canonical APOBEC SBS signatures SBS2 (TCA→TTA and TCT→TTT) and SBS13 (TCA→TGA and TCT→TGT). Experiments in cell culture showed that, in addition to single nucleotide substitutions, APOBEC3A induces indels, mainly TC→T (DeWeerd et al. 2022). In parallel, we discovered the corresponding 83-type indel signature, ID_A, which, compared to C_ID9, is more similar to the experimental indel spectrum in DeWeerd et al. (2022). Notably ID9 and InsDel9 in both the 89-type and 476-type classifications have many deletions of single thymines, while ID_A and InsDel_A in the 89-type and 476-type classifications lack these (Figure S2C). In addition, the InsDel9 476-type signature also has many deletions of a single C following A or G, while InsDel_A has almost exclusively deletions of a single C following T. This signature was also noted in Koh et al. (2025), who denoted it InD9a. We propose that this signature is distinct enough from InsDel9 in the 476-type classification to warrant an identifier.

MSS tumors: single-thymine indels in poly-T tracts

De-novo signature discovery using both HDP- and NMF in all 3 indel signature classifications identified signatures with similar proportions of deletions and insertions of a single thymine in poly-T tracts. These occur almost exclusively in MSS tumors. We termed these ID_N (the 83-type signature) plus InsDel_N and InsDel_N (the 89- and 476-type signatures). Previous analysis in the 83-type classification allocated insertions and deletions of single thymines in poly-T tracts to separate signatures ID1 and ID2, and did not differentiate between signature occurrence in MSS versus MSI-H (Alexandrov et al. 2020). We first focus on InsDel_N which occurs in 2,629 of 6,770 MSS tumors (39%) in diverse cancer types and in only 9 of 176 MSI-H tumors (5%, Table S10). Roughly half the tumors with InsDel_N also have InsDel1a, and InsDel1a also occurs without InsDel_N in 2,596 MSS tumors (39%) and 35 MSI-H tumors (20%). Thus, many MSS tumors have roughly equal proportions of thymine deletions and insertion in long poly-T tracts (InsDel_N), but other tumors have varying proportions of additional insertions (InsDel_1a), and some tumor spectra are completely dominated by these insertions. The other signature with approximately equal proportions of insertions and deletions of a thymine in a poly-T tract, InsDel_N , is rare in both MSS and MSI-H tumors, occurring in only 55 of 6,770 MSS tumors and 2 of 175 MSI-H tumors.

Indels in poly-T tracts and microsatellite contraction in MSI-H tumors

In addition to enrichment for InsDel2a, 2b, and 2c, which represent single-thymine deletions from poly-T tracts, MSI-H tumors were notably enriched for signatures InsDel7, D, and J (Figure 4a). InsDel7, like InsDel2a/b/c, is dominated by single-thymine deletions from poly-T tracts, but with a distinct distribution of sequences flanking the poly-T tract. InsDel_J consists almost entirely of deletions of single dinucleotides from a dinucleotide microsatellites, e.g. deletion of CA from (CA)⁵, or deletion of GT from (GT)⁸ [see also entries for the 476-indel-type category Del2:U2:R(5,) in Table S1]. Another signature, InsDel1a, which is dominated by single-thymine deletions into poly-T tracts occurred in 20% of MSI-H tumors. This is a reasonably high frequency, though substantially less than the 38% of MSS tumors (Table S10).

The new indel classifications reveal an accurate signature of RNase H2 deficiency

Cells with RNase H2 deficiency cannot efficiently remove ribonucleotides that had previously been incorporated into genomic DNA, and this sets the stage for topoisomerase-1 (TOP1) associated mutagenesis. This was previously proposed to generate indel signature ID4 (Reijns et al. 2022; Koh et al. 2025; Jin et al. 2024). However, we detected a new indel signature, InsDel_F, which better corresponds to RNase H2 deficiency experimental data than ID4 (Figure 6, Figure S5). The profile of deletions of 2 bases is very different between InsDel_F and ID4: InsDel_F has distinctively high proportions of (i) 2-base deletions with microhomology (almost all TCT → T or TGT → T) and (ii) 2-base deletions from paired 2-base doublets (most often CTCT → CT). InsDel4 has much lower proportions of these two deletion types and also has additional prominent deletion types that are essentially absent in InsDel_F. These additional deletion types are mainly deletions of 3 or 4 bases with microhomology and deletions of 3 or 4 bases from paired doublets (e.g. TCTTCT → TCT, GTTTGTTT → GTTT). The spectra in RNase H2 deficient cell lines consist of signatures due to mutational processes intrinsic to the parental cell line combined with high proportions of mutations characteristic of InsDel_F (Figure S5). Like InsDel_F, these cell-line spectra have 2-base deletions with microhomology and 2-base deletions from paired 2-base doublets. These spectra conspicuously lack the 3- and 4-base deletions present in InsDel4. Similarly, RNase H2 deficient mouse tumors have high proportions of the 2-base deletions characteristic of InsDel_F and lack the 3 and 4 base deletions present in InsDel4.

Associations between indel signatures and SBS signatures

We further analyzed the correlations between the indel signature activities we derived and the SBS signature activities from Degasperi et al. (2022) (Tables S11, S12). We first computed a matrix of correlations between the numbers of mutations attributed to SBS signatures the numbers attributed to indel signatures for all pairs of SBS indel mutational signatures. Agglomerative hierarchical clustering on the rows and columns of this matrix using 1 – the Pearson correlation coefficient as distance identified several clusters (Figure 4B). The APOBEC-related cluster includes SBS2 and SBS13 and showed moderate correlations with ID_A and InsDel_A. The homologous recombination deficiency (dHR) cluster, consisting of SBS3 and SBS8, was strongly associated with InsDel6 and InsDel9 (Figure 4B, “HR defects”). UV-related signatures, such as SBS7a, SBS38, InsDel13, and InsDel_H, also clustered together, reflecting their shared etiology. The cluster related to mutations in the proofreading domain of the polymerase epsilon (*POLE*) gene had strong links between SBS signature SBS10a and indel signature InsDel1b and InsDel_K. Additional analysis of tumor somatic mutations confirmed that these signatures are associated with dysfunctional *POLE*

proofreading plus, in some tumors, mismatch repair deficiency (Table S9 <add hypermutator info to legend>).

Within the mismatch repair deficiency (MMR) cluster, we observed a strong network of correlations between SBS6, SBS26, SBS44, and a group of indel signatures, including C_ID2, InsDel2b, InsDel2c, InsDel7, InsDel_D, and InsDel_J (Figure 4, “MMR defects”). Another notable group included InsDel_B, InsDel5a, and InsDel5b, which co-occurred with liver-cancer associated SBS signatures SBS12 and SBS16.

The 89-type taxonomy also allowed us to better resolve the relationships within previously broader categories. For instance, the three signatures derived from C_ID1 (InsDel1a, InsDel1b, and InsDel1c) each showed distinct patterns of correlation to SBS signatures (Figure 4B). InsDel1a and the 83-type signature C_ID1 had similar correlation profiles and frequently co-occurred with SBS88, SBS17, and SBS93, which are prominent in the GI enriched cluster. In contrast, InsDel1b was linked to *POLE* proofreading errors and InsDel1c was associated with SBS liver cancer signatures SBS12 and SBS16.

These correlation patterns provide insight into the possible origins of previously uncharacterized SBS signatures. For example, SBS92 closely mirrored the correlation profile of SBS4, with strong associations with C_ID3 and InsDel3b, further supporting a link with tobacco smoking (Figure 4B, “Tobacco Smoking”) (Lawson et al. 2020). In the gastrointestinal (GI) cluster, which includes signatures common in gastrointestinal tumors (SBS17, SBS18, SBS1, SBS88, and SBS93), we observed that C_ID18/InsDel18, ID_N/InsDel_N were associated with SBS signatures typically seen in the GI tract. SBS93, in particular, shows a strong transcriptional strand bias <what is the reference for this>, suggesting its association with specific DNA adducts in the GI tract and a possible connection to C_ID14/InsDel14. In addition C_ID1 and InsDel1a had stronger correlations with SBS1 and SBS18, supporting their association with mutagenesis driven by reactive oxygen species (Figure 4).

Topography of indel mutational signatures <STEVE REVIEW>

We evaluated the interplay between our the 83-type signatures that we identified and several genomic topographical features. Transcription-coupled nucleotide excision repair was known to cause transcription strand asymmetries, since DNA bulky adducts on the transcribed strand will be preferentially repaired in the transcription active region across the genome (add cite). Transcription strand asymmetries were shown in 17 out of 33 83-type signatures (Figure 5A, Figure S3A, Table S13). Signatures attributed to exogenous mutational processes, such as C_ID3 (tobacco smoking exposure), C_ID14 (correlated with SBS93) and C_ID18 (colibactin exposure), showed consistent transcription strand bias with mutations enriched in the transcribed strand. Another exogenous mutational signature C_ID13, which is attributed to UV exposure, showed enrichment in the un-transcribed strand. This is due to the damage caused by UV happened on cytosine instead of guanine. In 83-type signatures associated with defective endogenous mutational processes, enriched mutations on the un-transcribed strand were observed for C_ID1 (slippage during DNA replication) and C_ID5 (clock-like signature), while enriched mutations on the transcribed strand were observed for ID_F (topoisomerase-1 [TOP1] associated mutagenesis stemming from RNase H2 deficiency). Interestingly, 3 defective MMR signatures also showed transcription strand asymmetries in different directions, with two of them (C_ID7 and ID_K) having enriched mutations on the transcribed strand, and one of them (ID_N) having enriched mutations mainly on the un-transcribed strand.

Replication strand asymmetries were observed in 12 out of 33 83-type signatures (Figure 5B, Figure S3B, Table S13). Analogous to transcription strand asymmetries, the majority of signatures attributable to exogenous mutagenic exposures or defective endogenous repair processes exhibited pronounced replication strand bias, manifesting preferentially on either the leading or lagging strand. ID_K, which is enriched for mutations causing POLE proofreading defects, was consistently enriched for leading strand mutations. This is consistent with SBS signatures SBS10a and SBS10b, which are also, driven by POLE proofreading defects, thus further supporting a link between POLE proofreading defects and leading-strand mutagenesis.

We also examined the mutation enrichment of 83-type signatures in genic and intergenic regions (Figure S3C). Most signatures showed mutation bias towards intergenic regions, while 6 signatures showed enrichment in genic regions (Figure 5C). Among the 6 signatures, 3 of them were associated with defective endogenous mutational processes, including C_ID8 (repair of DNA double strand breaks by NHEJ), C_ID17 (TOP2A K743N mediated mutagenesis) and ID_F (TOP1-mediated mutagenesis). Another 3 signatures with mutations enriched in genic regions were of unknown etiology (C_ID4, ID_E and ID_L).

The effect of DNA replication timing was also observed in many 83-type signatures (Figure 5D, Figure S4, Table S14). Most signatures were consistently enriched in late-replicating regions across different cancer types, while only two signatures, C_ID10 and C_ID17 (TOP2A K743N mediated mutagenesis), showed enrichment in early-replicating regions. Four signatures were unaffected by replication timing in the majority of cancers presenting this signature, including C_ID1 (Slippage during DNA replication), C_ID5 (Clock-like signature), C_ID18 (Colibactin exposure), and two defective MMR signatures (ID_7 and ID_K). Interestingly, several signatures showed cancer-specific enrichment in replication timing, including C_ID8 (Repair of DNA double strand breaks by NHEJ) and ID_F (TOP1-mediated mutagenesis). For example, ID_F showed enriched mutations in late replication regions in bladder cancer, while in skin cancer it showed the opposite trend. For 89-type signatures, we conducted similar analysis without simulation data. The majority of signatures showed enrichment in late-replicating regions, which is consistent with what we observed 83-type signatures (Figure S4, Table S15). InsDel_L, the corresponding signature of ID_L, showed the same stomach cancer specific enrichment in early replication regions, which strengthen the correlation between 83-type and 89-type signatures.

Demographic Associations with Gender and Age STEVE REVIEW

To explore potential gender biases in mutational processes, we systematically compared signature activities between males and females using two-sided Fisher's exact tests. After applying Benjamini-Hochberg adjustment ($q < 0.2$), nine significant gender-associated signatures were identified, underscoring a striking polarity across tumor types (Figure S6A, Table S16). <parallel structure>Male-enriched mutational processes predominated in lung, liver, and "Other" cancer categories. Specifically, lung cancer showed significant male association with InsDel3b (OR = 0.367, $q = 0.061$) and C_ID3 (OR = 0.397, $q = 0.188$), signatures previously linked to tobacco exposure, suggesting that higher smoking-related mutational burdens contribute to this enrichment among men. In liver cancer, a similar male predominance was observed for InsDel5a (OR = 0.481, $q = 0.118$; clock-like) and InsDel8/C_ID8 (OR = 0.271, $q = 0.184$; associated with DSB repair by NHEJ and age). <mentioned anywhere else?>The "Other" tumor group exhibited male bias in C_ID19 (OR = 0.424, $q = 0.188$) and InsDel19b (OR = 0.448, $q = 0.197$). In contrast, female-enriched signatures were found in esophageal and lymphoid malignancies, most notably in esophageal cancer

for InsDel1a (OR = 3.85, $q = 0.061$) and C_ID1 (OR = 3.22, $q = 0.188$), as well as InsDel1c (OR = 2.63, $q = 0.194$) in lymphoid cancers. No additional signature–cancer type combinations reached a q -value below 0.2. Collectively, these findings suggest that specific mutational processes and their underlying biological processes might be modulated by gender, shaping distinct patterns of cancer susceptibility and progression.

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The clock-like association was described in the following paragraph. We can either not mention the clock-like in this part, or bring the age-association part earlier.

To characterize the “clock-like” behavior of these signatures, we examined correlations between signature activity and patient age at diagnosis using Spearman correlation analyses (Figure S6B). We found that C_ID1, InsDel1a, C_ID5, and InsDel5a showed robust, age-associated “clock-like” mutagenesis in multiple cancer types, as well as across the overall genome. More specifically, C_ID5 and InsDel5a correlated with age in prostate, pancreas, liver, and kidney cancers, while C_ID1 and InsDel1a were age-associated in breast, prostate, and pancreas cancers. Other clock-like signatures demonstrated pronounced tissue specificity; for example, InsDel9 showed a more attenuated relationship between age and mutational accumulation in prostate cancer, while InsDel10 displayed clear clock-like properties in pancreatic cancers. Additionally, some signatures, such as InsDel5b, InsDel5c, and C_ID4/InsDel4, were correlated with age when considering all tumor types collectively but did not show significant associations in any single cancer type (Table S17). However, these broad pan-cancer associations likely reflect weak or heterogeneous age dependencies spread across multiple tumor types, rather than strong, tissue-specific age effects.

Taken together, these analyses reveal a demographic landscape of mutational mechanisms, indicating that genomic instability is sculpted by both sex-specific biological influences and tissue-dependent aging processes.

Discussion

In this study, we analyzed somatic indels in 6,975 whole-genome tumor samples spanning 36 cancer types and established a comprehensive repertoire of indel mutational signatures across the 83-type, 89-type, and 476-type classification systems. By combining de novo signature discovery with systematic cross-classification mapping, we show that the 89-type and 476-type taxonomies provide substantially greater resolution than the conventional 83-type scheme, particularly for single-base insertions and deletions in context-dependent sequence environments. This expanded framework not only clarifies correspondences among previously reported signatures, but also enables the identification of novel signatures, including ID_F and InsDel_F, which are associated with TOP1-related mutagenesis and RNase H2 deficiency. In addition, we identified six 89-type and 476-type signatures, together with four 83-type signatures, that are strongly enriched in high-mutation-burden tumors, and further refined the interpretation of indel processes linked to mismatch repair deficiency and/or POLE/POLD proofreading dysfunction.

The recently developed 89-type taxonomy by Koh et al. provided a new framework for classifying indels, particularly facilitating finer resolution of 1 bp T insertions and deletions in different contexts, and enabled more nuanced exploration of the genetic consequences of MMR defects. Koh et al., built

a 37-signature indel database was derived from 4,775 genomes across seven cancer types. However, challenges remain in connecting the nomenclature of the 89-type signatures to the established 83-type system, as well as in translating between these classification schemes.

A major strength of this study is the systematic mapping between the conventional 83-type scheme and the newer 89-type and 476-type indel taxonomies. Because translation across these classification systems is inherently imperfect, direct comparison of signatures is not straightforward. By leveraging tumors dominated by single mutational processes, we established robust correspondences among signatures identified in all three frameworks. This unified mapping not only improves continuity with legacy studies based on the 83-type classification, but also provides a practical reference structure that will facilitate future indel signature analyses using the more informative 89-type and 476-type schemes.

The new indel framework also refined the interpretation of several biologically important mutational processes. Notably, the new TOP1-associated signatures, ID_F and InsDel_F, were more consistent with RNase H2 deficiency than previously recognized indel patterns. The enrichment of microhomology-mediated deletions and paired doublet deletions in these signatures supports a mechanistic link between RNase H2 dysfunction and a characteristic indel spectrum. Similarly, we identified multiple signatures strongly associated with microsatellite instability, and the finer resolution of the new taxonomies allowed us to distinguish MSI-related signatures that were previously aggregated or less clearly resolved. Given the clinical relevance of MSI and mismatch repair deficiency, this improved annotation may have direct utility for tumor classification and therapeutic stratification.

Our integrative analyses also examined the relationships between indel signatures and SBS signatures. The clustering of APOBEC-, UV-, HR-, MMR-, and POLE-related signatures is consistent with shared or linked mutational processes across mutation classes, supporting the idea that indel mutational signatures often arise in the context of broader mutational programs. These associations provide an additional layer of biological interpretation and help connect indel signatures to established mutagenic pathways.

We also investigated the genome-wide topographical features associated with indel signatures, including transcriptional strand bias, replication strand bias, genic distribution, and replication timing. Notably, because the COSMIC catalog does not provide such topographical annotations for many indel signatures, our analysis in this large cohort helps fill an important gap in the current knowledge base. The observed associations highlight the influence of DNA repair and replication dynamics on indel formation and suggest that genomic context contributes to the emergence and distribution of distinct indel patterns. Together, these analyses provide a more comprehensive view of the biological determinants of indel mutagenesis.

i Comment 1, Unknown Author, 2026-07-02

Reply to Mo Liu (05/29/2026, 16:03): “...” Hi Steve, I summarized the topographical annotations available on the COSMIC website. In our paper, we examined the following four features: replication strand asymmetry, transcription strand asymmetry, genic strand asymmetry, and replication timing. On COSMIC, except for ID6, no signature was fully annotated with all four features. So our work indeed provides important topographical annotations for the 83-type signatures. I’ve summarized the information in the attached file. Please let me know if you need more information. Thanks, Mo See COSMICv3.6 ID topographical annotation.xlsx

i Comment 2, Mo Liu, 2026-05-29

I update via email.

i Comment 3, Steve Rozen, Ph.D., 2026-05-28

Which ones do we supply that COSMIC does not, and are there any discordances between what we get and COSMIC for signatures that COSMIC does have topographical annotations for?

i Comment 4, Mo Liu, 2026-05-25

This is true. But not sure if this worths mentioning?

A potential source of technical variation in our analysis is the use of data generated by different variant-calling pipelines used for PCAWG and Hartwig cohorts. We observed a higher frequency of indels occurring in sequence contexts containing more than nine repeat units in the Hartwig data, raising the possibility that differences in variant calling, local realignment, or indel representation contributed to this discrepancy. To assess and minimize the potential impact of this bias, we performed two additional analyses. First, we excluded indels associated with more than nine repeat units to improve comparability with the PCAWG calling pipeline. Second, for indels occurring in repetitive sequence contexts, we normalized their representation by moving each indel to the leftmost possible position within the sequence context. Thus, we cannot exclude residual caller- or cohort-specific effects, particularly for indels in highly repetitive sequence contexts and for signatures supported by relatively few mutations. Nevertheless, the overall consistency of the dominant signatures across datasets, together with the robustness of the results after filtering and representation normalization, suggests that variant-calling bias is unlikely to substantially affect the main conclusions of this study.

i Comment 5, Mo Liu, 2026-09-06

New paragraph added by Mo to discuss the potential caller batch effects

Importantly, the 89-type system is not uniformly superior in all settings. Although it improves resolution for indels occurring in long poly-T tracts, it also compresses indels of length greater than or equal to 2 bp, and in some cases may merge biologically distinct patterns that are separated in the 83-type framework. For example, the relationships between InsDel_C and InsDel13, as well as between InsDel11 and InsDel16, suggest that some signatures with distinct sequence preferences in the 83-type system are represented together in the 89-type taxonomy. By analyzing a larger and more diverse cancer genome cohort, we not only expanded the existing indel signature database, but also systematically linked and unified the nomenclature of the 83-type and 89-type signatures. Taken together, these results suggest that although the 89-type taxonomy represents an important advance, further refinement of indel classification will be needed to fully capture the diversity of indel mutational processes. The unified framework presented here should therefore support more consistent interpretation of current signatures and provide a foundation for future development of an even more informative indel taxonomy.

Materials and methods

Data sources

Somatic indel calls from the ICGC/TCGA (International Cancer Genome Consortium/The Cancer Genome Atlas) Pan-Cancer Analysis of Whole Genomes Consortium and associated clinical characteristics were obtained from the ICGC data portal (<https://dcc.icgc.org/releases/current/Projects/>, which has been retired. The data were downloaded on 9 May, 2024). Hartwig Medical Foundation (Hartwig) through standardized procedures and request forms that can be found at <https://www.hartwigmedicalfoundation.nl/en/applying-for-data/>. Clinical traits such as cancer type, age and gender of the Hartwig genomes were found from supplementary files of Priestley et al. (2019). <we can duplicate this in the code but we don't need to re-present the Priestly table> These data was also provided are available in Table S16 in Priestley et al. (2019).

De novo Mutational signature extraction

De novo mutational signature extraction was performed using two complementary tools applied to the 83-type, 89-type, and 476-type mutational spectra. A stratified strategy was adopted to capture rare and cancer-specific signatures, encompassing analyses of all genomes, specific cancer types, high-TMB samples, and the PCAWG and Hartwig cohorts individually.

For the hierarchical Dirichlet process-based approach, mSigHdp (v2.1.2) was used. When analyzing all samples, cancer type was used to define the hierarchical structure of the model. For analyses restricted to individual cancer types or to genomes with high tumor mutational burden, *de novo* signatures were extracted using two-layer hierarchical Dirichlet process (HDP) mixture models. In all mSigHdp analyses, the following parameters were applied: seedNumber = 1234, burnin = 1000, burnin.multiplier = 20, post.n = 200, post.space = 100, num.child.process = 20, gamma.alpha = 1, and gamma.beta = 50. For analyses including all samples or genomes with high tumor mutational burden, an additional downsampling step was applied (downsample_threshold = 3000) to reduce computational time.

In parallel, *de novo* mutational signatures were extracted using the Non-negative matrix factorization based approach (NMF), SigProfilerExtractor (v1.1.24). For analyses including all genomes, NMF was performed with the number of signatures evaluated between 10 and 40 (minimum_signatures = 10; maximum_signatures = 40). For analyses restricted to individual cancer types or to genomes with high tumor mutational burden, the number of signatures evaluated ranged from 2 to 20 (minimum_signatures = 2; maximum_signatures = 20). Each factorization was repeated 100 times (nmf_replicates = 100), with all other parameters set to their recommended default values. Most signatures were extracted at the “Optimal Solution” identified by SigProfilerExtractor, although several rare signatures were identified in alternative solutions.

Match 83-type and 83-type signatures into reference signatures

For 83-type signatures, the signatures were matched to previously identified COSMIC signatures (v3.5, <https://cancer.sanger.ac.uk/signatures/id/>). For 89-type signatures, the signatures supported by both mSigHdp and SigProfilerExtractor were matched to the signatures reported in Koh et al. A detailed comparison can be found in Table 1.

We compared all *de novo* signatures supported by both mSigHdp and SigProfilerExtractor to the published signatures and categorized them into three groups: (1) known signature: if an extracted signature has a cosine similarity of ≥ 0.85 with a COSMIC signature; (2) merged signatures: if an extracted signatures can be reconstructed by at most 3 COSMIC signatures with a reconstructed similarity of ≥ 0.9 ; (3) novel signatures: the extracted signatures do not fit into the known signatures or the merged signatures. We focused on (1) and (3) in this study.

Assignment of signature activity to each individual genome

To leverage the resolution of both the 83-type and 89-type mutational signatures, we constructed a unified 172-type reference set and applied SigProfilerAssignment to decompose the 172-type spectra in low-TMB tumors (indels $< 5,000$). In contrast, high-TMB tumors (indels $\geq 5,000$), as well as tumors where 1-bp T insertions or deletions in at least five poly-T tracts account for $\geq 50\%$ of the mutations (Table S18), often exhibited highly sparse spectra. This sparsity further masks other mutational signals. Therefore regarding these cases, we employed mSigAct, estimating likelihoods under a multinomial distribution. This was performed in a two-step computational framework. First, we applied the `find_best_reconstruction_QP` function from the mSigTools R package (v1.0.7), which performs fast signature attribution via quadratic programming optimization. Second, we refined these initial estimates using the `PresenceAttributeSigActivity` function from the mSigAct R package. Final signature assignments were obtained by integrating results from SigProfilerAssignment for low-TMB tumors and mSigAct for high-TMB tumors.

Simulating synthetic cancer datasets

We generated synthetic cancer datasets in 83-type indels using SigProfilerSimulator (<https://bmcbioinformatics.biomedcentral.com/articles/10.1186/s12859-020-03772-3>).

Annotating somatic indels based on transcribed versus un-transcribed strand

We followed the method in (<https://doi.org/10.1016/j.celrep.2023.112930>). Briefly, somatic indels were called with respect to the + strand of the reference genome and further annotated in regard to the pyrimidine base(s) of the insertion/deletion. Thus, indels with only C or T bases were annotated as + strand mutations; indels with only A or G bases were annotated as - strand mutations. The remaining indels were not included in the analysis. Next, + strand indels in protein coding genes were further subclassified as transcribed (template) if the gene's sense strand was on the + strand of the genome, or else un-transcribed (sense). The logic was inverted for - strand indels. Indels in bidirectionally transcribed regions were ignored.

Annotating somatic indels based on leading versus lagging replication strand

Replication strand was determined by wavelet-smoothed replication-timing signal data that indicated both “valleys” (replication termination zones) and “peaks” (replication initiation zones) (<https://hgdownload.cse.ucsc.edu/goldenPath/hg19/encodeDCC/wgEncodeUwRepliSeq/>). Valleys and peaks were sorted by the genomic coordinate in ascending order. In regard to + strand of the reference genome, replication timing signals were examined for consecutive stretches of the genome

(from valley to peak or from peak to valley), with positive slope corresponded to leading strand regions and negative slope corresponded to lagging strand regions. Then for the - strand of the reference genome, leading regions (- slope) and lagging regions (+slope) were automatically acquired. Similar to the annotation for transcription, indels were first annotated as + or - strand mutations based on the pyrimidine bases. Next, indels were counted as being on leading strand or lagging strand based on their occupancy in a leading or lagging region.

Detecting strand asymmetries across cancer types

Strand asymmetry analyses were based on the assignment of signature probabilities to each individual indel mutation. Only indels with the probability greater than or equal to 0.50 to a certain indel signature were retained. For each indel signature and for all cancer types having this mutational signature, we retrieved the number of indels on each strand/region. In strand asymmetries analyses, only cancer types with at least 1,000 somatic mutations unambiguously attributed to an individual mutational signature were included.

For each strand asymmetry analyses (genic and intergenic region asymmetry, transcription strand asymmetry, replication strand asymmetry), indel mutations were split into two types and counted (genic vs intergenic mutation, leading strand vs lagging strand mutation, un-transcribed strand vs transcribed strand mutation). The two types were denoted as +/- strand mutations strand mutations in all three cases. The ratio of real somatic indels and the ratio of simulated somatic indels was calculated separately:

$$\text{strand ratio} = (+ \text{ strand_mutation_counts}) \cdot (- \text{ strand_mutation_counts})^{-1}$$

We calculated the osdds ratio between the strand ratio of real somatic indels and the strand ratio of simulated somatic indels.

We then calculated p values calculated for the odds ratio using Fisher's exact test. Only strand asymmetries with p value > 0.05 were considered showing strand asymmetries.

For a given cancer type, for e.g. genic vs non-in a sup table

genic non-genic

real 100 600

syn 100 300

Analyses of replication timing across cancer types

Replication timing data were obtained from (<https://genomebiology.biomedcentral.com/articles/10.1186/s13059-018-1509-y>) The replication time signals were sorted in a descending order and subsequently divided into deciles. Somatic indels were counted within the corresponding deciles based on their overlap with the replication domains in the examined deciles. As with other analyses, for each individual indel signature, the reported replication timing analyses included only cancer types with at least 1,000 somatic mutations unambiguously attributed to it.

Replication timing mutation counts were generated for both real and simulated somatic indels. To classify whether the replication timing mutation density was increasing, flat, or decreasing, two linear regression models were fitted to the values of the real somatic indels count $R = (R_1, R_2, R_3,$

... R_{10}) and the values of simulated somatic indel counts $S(X_1, X_2, X_3, \dots, X_{10})$, respectively. The two models are $lm(R \sim c)$ and $lm(S \sim c)$, where replication timing denotes a vector $c = (1, 2, 3 \dots 10)$.

The replication timing trend for an 83-type signature was determined for both real and simulated data. An indel signature was considered to be generally unaffected by replication timing if the slope m was not statistically significant from a flat line. Otherwise, with the slope m statistically significant from a flat line, an ID signature was considered to be increasing from early to late replicating regions if the slope $m > 0$, and was considered to be decreasing from early to late replicating regions if the slope $m < 0$.

If the trends of a certain 83-type signature for the two datasets (real and simulated data) were different, the relative trend for real data comparing the simulated data was the final trend for the 83-type signature. Otherwise, if the trends of a certain 83-type signature for the two dataset were the same, a third multiple linear regression model was fitted: $lm(R \sim S + c)$

Similarly, the final trend of an 83-type signature were determined by the significance of the coefficient of replication timing. If it is not significant, an 83-type signature was considered to be generally unaffected by replication timing. Otherwise, if the coefficient of replication timing > 0 , an 83-type signature was considered to be increasing from early to late replicating regions; if the coefficient of replication timing < 0 , an 83-type signature was considered to be decreasing from early to late replicating regions.

MSI/MSS status and high/low TMB status

For PCAWG genomes, the MSI (microsatellite instability) status was evaluated by the PCAWG working group and obtained from the synapse repository (<https://www.synapse.org/#!Synapse:syn8016399>, the data was downloaded on May 2022). For Hartwig genomes, the MSI status was downloaded from the supplementary data of Priestley et al., 2019. The genomes with $> 5,000$ IDs were labelled as high TMB tumors. The thresholds were selected based on the minimum number of mutations of the pre-defined MSI tumors. We then used MSI-seq to predict the MSI status of high TMB tumors.

Extended sequence context analysis

To analyze a specific signature and indel type of interest, we first identified the 5 genomes with the highest contribution of the corresponding signature activity. From these genomes, we extracted all indels of the relevant type. We then examined the nucleotide sequence within a 21-base pair window centered on each indel site (± 10 nucleotides from the indel position). For each position within this window, we calculated the frequency of each nucleotide (A, T, C, and G). The logo was plotted based on the frequency matrix by seqLogo function of seqLogo R package (version 1.71.0)

Funding

The work was supported by the Young Scientists Fund of the National Natural Science Foundation of China (Grant No. 32500546), Natural Science Foundation of Guangdong Province, China (Grant No. 2026A1515010445), International Science and Technology Innovation Cooperation between

Governments for National Key R&D Program Projects (2023YFE0107700), Guangzhou Municipal Science and Technology Bureau (2025A04J4304), Shenzhen Science and Technology Innovation Program (JCYJ20250604174809012).

Author contributions

M.L. and S.G.R. conceived the project and designed the analyses. M.L. and S.G.R. designed computational analysis. M.L., Q.Z. and Y.Y. performed mutational signature extraction, attribution and characterization computational analysis. M.H., and R.Y. characterized the topography of 83-type mutational signatures. M.L., X.M.W., R.S. and S.G.R. designed the website. Data interpretation and manuscript write-up were carried out by M.L., M.H., and S.G.R., with contributions from all other authors. All authors reviewed and approved the final manuscript.

Declaration of generative AI and AI-assisted technologies in the writing process

During the preparation of this work the author(s) used Claude in order to polish the text and ensure grammatical accuracy and appropriate vocabulary usage. After using this tool/service, the author(s) reviewed and edited the content as needed and take(s) full responsibility for the content of the published article.

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Table 1: Information of the 89-type indel signatures and their corresponding 83-type signatures. This table summarizes the correspondence between the 89-type indel signatures and signatures from the 476-type collapsed set, the 89-type signatures reported in Koh et al., and the 83-type signatures reported in Jin et al. For each 89-type signature, we report the cosine similarity to the corresponding collapsed 476-type signature spectrum, the cosine similarity between the 89-type signature and its best-matching spectrum, the closest matching signature from Koh et al. and its cosine similarity, and the closest matching signature from Jin et al. and its cosine similarity.

89-Type Signature ID	Cosine 476-Type Sig Collapsed to 89-Type	Cosine 89-Type Sig vs Best Match Spectrum	Koh et al. Sig	Cosine 89-Type Sig vs Koh et al.	83-Type Sig ID	Jin et al. Sig	Cosine vs Jin et al.
InsDel1a	0.9923	0.9831	InD14	0.9023	C_ID1	ID1*	0.9849
InsDel1b	0.9925	0.9850	InD15	0.9796	C_ID1	ID1	0.9849
InsDel1c	0.9819	0.9675	InD14*	0.9415	C_ID1	ID1	0.9849
InsDel2a	0.9853	0.9710	InD2a	0.9824	C_ID2	ID2*	0.9932
InsDel2b	0.9858	0.9645			C_ID2	ID2	0.9932
InsDel2c	0.9842	0.9744			C_ID2	ID2	0.9932
InsDel3a	0.9515	0.9738			C_ID3	ID3*	0.9403
InsDel3b	0.9970	0.9952	InD3b	0.9527	C_ID3	ID3	0.9403
InsDel4	0.9955	0.9845	InD4a	0.9820	C_ID4		
InsDel5a	0.9983	0.9908	InD29		C_ID5		
InsDel5b	0.9953	0.9826			C_ID5		
InsDel5c	0.9575	0.9344			C_ID5		
InsDel6	0.9981	0.9949	InD6	0.9745	C_ID6	ID6	0.9156
InsDel7	0.9977	0.9967	InD7	0.9797	C_ID7**		
InsDel8	0.9942	0.9914	InD8	0.9812	C_ID8	ID8	0.9683
InsDel9	0.9992	0.9259	InD9b	0.9170	C_ID9		
InsDel10	0.9569	0.8457			C_ID10**	ID10	0.9016
InsDel11	1.0000	1.0000			C_ID11	ID11a	0.9975
InsDel12	0.9908	0.9701			C_ID12	ID12	0.9368
InsDel13	0.9962	0.9908	InD13*	0.9886	C_ID13	ID13	0.9580
InsDel14	0.9881	0.9945	InD12	0.9831	C_ID14	ID14	0.9584
InsDel15	1.0000	0.9972			C_ID15		
InsDel16	1.0000	1.0000			C_ID16		
InsDel17	1.0000	0.9999			C_ID17		
InsDel18	0.9994	0.9958	InD18	0.9959	C_ID18	ID18	0.9087
InsDel19a	0.9913	0.9494			C_ID19		
InsDel19b	0.9943	0.9922			C_ID19		
InsDel23	0.9945	0.9606			C_ID23		
InsDel_A α	0.9972	0.8097			ID_A		
InsDel_A β	0.9792	0.9361	InD9a	0.9650	ID_A		
InsDel_B	0.9919	0.9872			ID_B		
InsDel_C	0.9968	0.9713	InD13	0.9268	ID_C		
InsDel_D	0.9746	0.9879	InD1	0.9256	ID_D		
InsDel_E	0.9879	0.9776			ID_E		
InsDel_F	0.9993	0.9925			ID_F	ID4	0.9726
InsDel_G	0.9966	0.9547			ID_G	ID11b	0.9467
InsDel_H	0.9540	0.8206			ID_H		
InsDel_I	0.9991	0.9998			ID_I	ID25	0.9323
InsDel_J	0.9753	0.5628			ID_J**		

(continued)

89-Type Signature ID	Cosine 476-Type Sig Collapsed to 89-Type	Cosine 89-Type Sig vs Best Match Spectrum	Koh et al. Sig	Cosine 89-Type Sig vs Koh et al.	83-Type Sig ID	Jin et al. Sig	Cosine vs Jin et al.
InsDel_K	0.9964	0.8572	InD20	0.9488	ID_K	ID26	0.9815
InsDel_L	0.9981	0.9658			ID_L		
InsDel_M	0.9942	0.9999			ID_M		
InsDel_N α	0.9900	0.9840	InD16b	0.9229	ID_N		
InsDel_N β	0.9949	0.9850	InD21	0.9047	ID_N		

* This is the signature with the best match to the given type-89 signature.

** indicates that in the linking tumor's spectrum, insertions of single Ts in long poly-T contexts were set to 0 before calculating cosine similarity to the 83-type signature.

bar representing a the deletion of a 2-base unit from the 2-base unit duplicated (e.g. a 2-base unit TA, in the duplicated 2-base unit TATA, with the deletion being TATA→TA). The classification does not capture information on the bases involved. In the 89-type classification, these are captured in the tall red bar labeled “many possibilities”, which represents a wide range of deletions, up to deletions such as ATATATAT→AT, which are essentially absent in this signature. The tallest of the corresponding bars in the 476-type classification is labeled “e.g. TATA→TA, e.g. TGTG→TG”, in one-to-one correspondence with the the light orange bar in the 83-type classification with the same label. Like the 83-type classification, the 476-classification does not capture the identity of the bases involved for these deletions.

(C) At the far right, in the 83-type-classification, the light purple bar indicates 2-base deletions with microhomology, e.g. GAG→G, where the G flanking the deletion is the microhomologous base. The bases involved are not captured, and other possibilities include GTG→G, TAT→T, and many others. In the 89-type classification these and many other possible deletions are represented by the peak “Many Mh possibilities”, which includes, in addition, deletions such as ATCGTA→A, which are essentially absent from this signature. In the 476-type classification, as in the 83-type classification, the highest bar is labeled “2 base deletions with microhomology”, and it represents the same set of mutations as the corresponding bar in the 83-type classification. However, in general, even for the 476 classification there is many-to-many mapping of deletions and insertions of more than 1 base between the 83-type and 476-type classifications (Table S1).

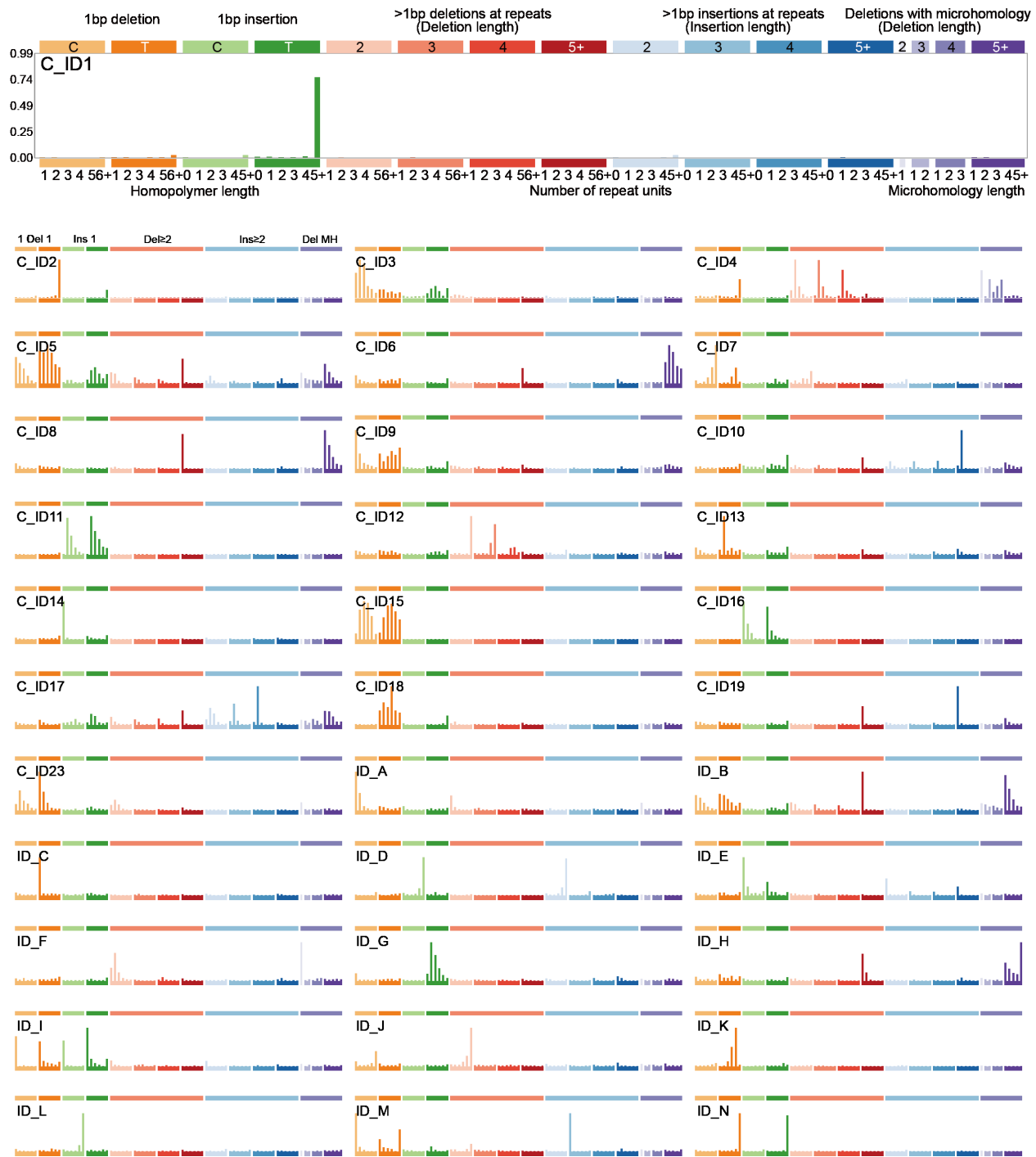


Figure 2: Profiles of signatures extracted in the 83-type classification scheme

The x-axes represents indel categories and the y-axes indicate the proportion of each indel category within each signature. Signatures C_ID1 through C_ID23 match reference signatures in COSMIC 3.5 and follow the same numbering. For example C_ID1 matches ID1. The remaining signatures, ID_A through ID_O were not present in COSMIC 3.5.

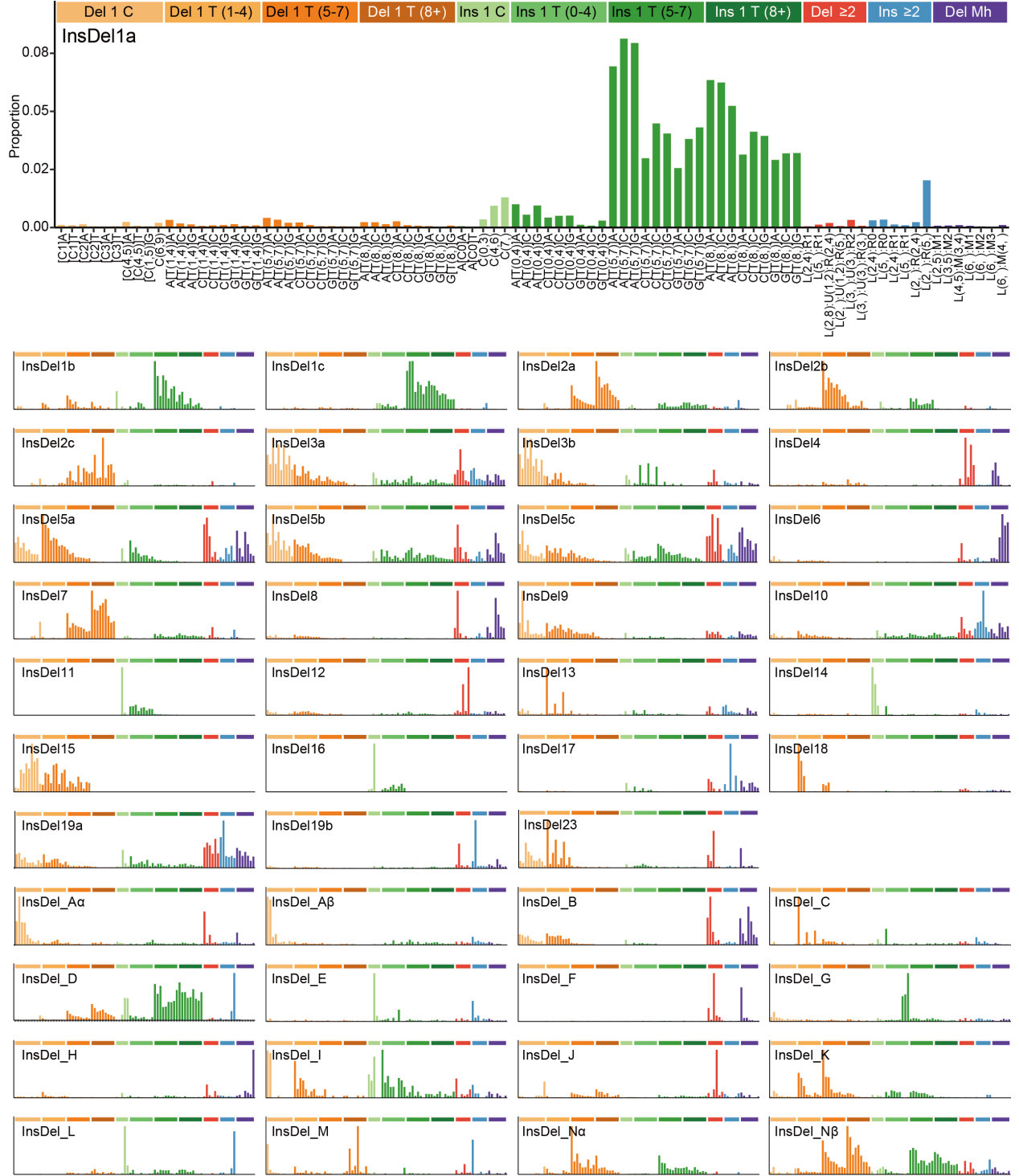


Figure 3: Profiles of signatures extracted in the 89-type classification scheme

The x-axes represent indel categories and the y-axes indicate the proportion of each indel category within each signature. For signatures that correspond to reference signatures in we use the same numbering and add suffixes (a, b, c, d) when a single COSMIC signature maps to multiple signatures in the 89-type classification. The identifier prefix “InsDel” distinguish these signatures from signatures in COSMIC (prefix “ID”) and Koh et al., 2025 (prefix InD).

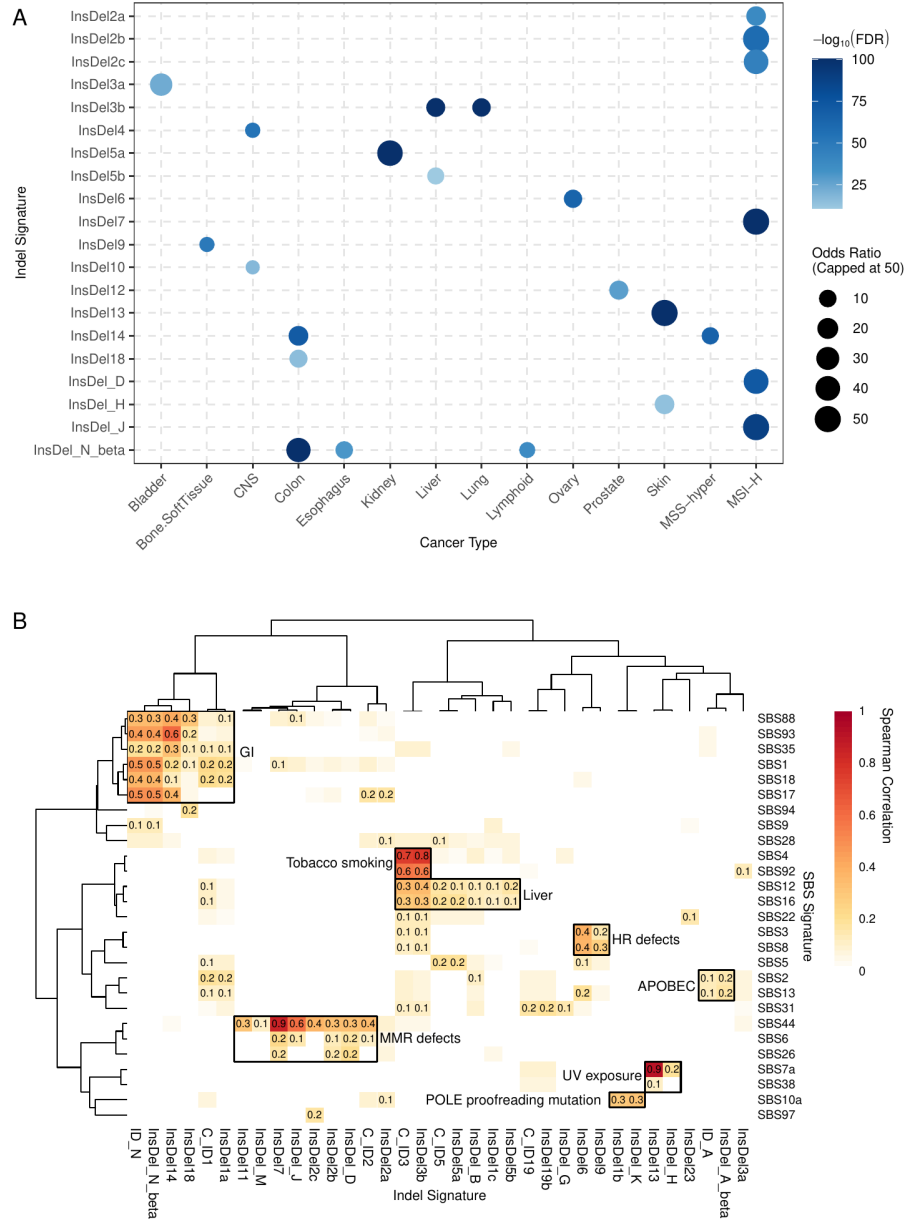


Figure 4: Association of indel signatures with cancer types and SBS signatures and cancer types

(A) Enrichment for presence of 89-type signatures across cancer types, plus hypermutated ($> 4,000$ indels) microsatellite stable (MSS) tumors and MSI-high (MSI-H) tumors. Only cancer types with odds ratios > 6 are shown. Benjamini-Hochberg false discovery rates based on p values from Fisher's exact test of number of tumors with the signature versus the number without in a given tumor type versus all other tumors.

(B) The heatmap of Spearman correlation coefficients between indel signatures and various SBS signatures. Signatures were excluded if all correlation coefficients were between -0.1 and 0.1 . Unsupervised hierarchical clustering of both rows and columns based on signature proportions identified 8 clusters, boxed and labeled in the figure. GI: Gastrointestinal.



Figure 5: 83-type mutational signatures asymmetries in strands, replication timing and DNA regions.

(A) Enrichment of mutations in genic and intergenic regions for 83-type signatures. Each row

represents one 83-type signature, where n reflects the number of cancer types in which the signature was found. Signatures consistently enriched in genic and intergenic regions with p values > 0.05 (Fisher’s exact test) are shown in circles with yellow and light blue colors, respectively. Grey colors indicate inconsistent enrichments in different cancer types. Columns display the odds ratio between the ratio of real mutations and the ratio of simulated mutations (See Methods). Only odds ratios above 1.10 are shown. Circle sizes reflect the proportion of cancer types exhibiting specific enrichment in DNA regions in a signature.

(B) Replication strand asymmetries of 83-type signatures. Data are presented in a format similar to the one in panel (A), with green and orange colors indicating replication strand asymmetries on the lagging and leading strands, respectively.

(C) Transcription strand asymmetries of 83-type signatures. Data are presented in a format similar to the one in panel (A), with red and dark blue colors indicating transcription strand asymmetries on the transcribed and untranscribed strands, respectively.

(D) 83-type signatures associated with early and late replication timing. Mutation densities of 83-type signatures per decile (y axes) are presented for early (left) to late (right) replication domains. Real mutations for signatures are shown as bars, and simulated mutations are shown as red dashed lines. As shown on top of each plot, green bars indicate signatures consistently associated with late replication timing across cancer types (e.g., C_ID3); yellow bars indicate signatures consistently associated with early replication timing across cancer types (e.g., C_ID17); purple bars indicate signatures not affected by replication timing across cancer types (e.g., ID_K); blue bars indicate signatures showing inconsistent trend across cancer types (e.g., ID_F)..

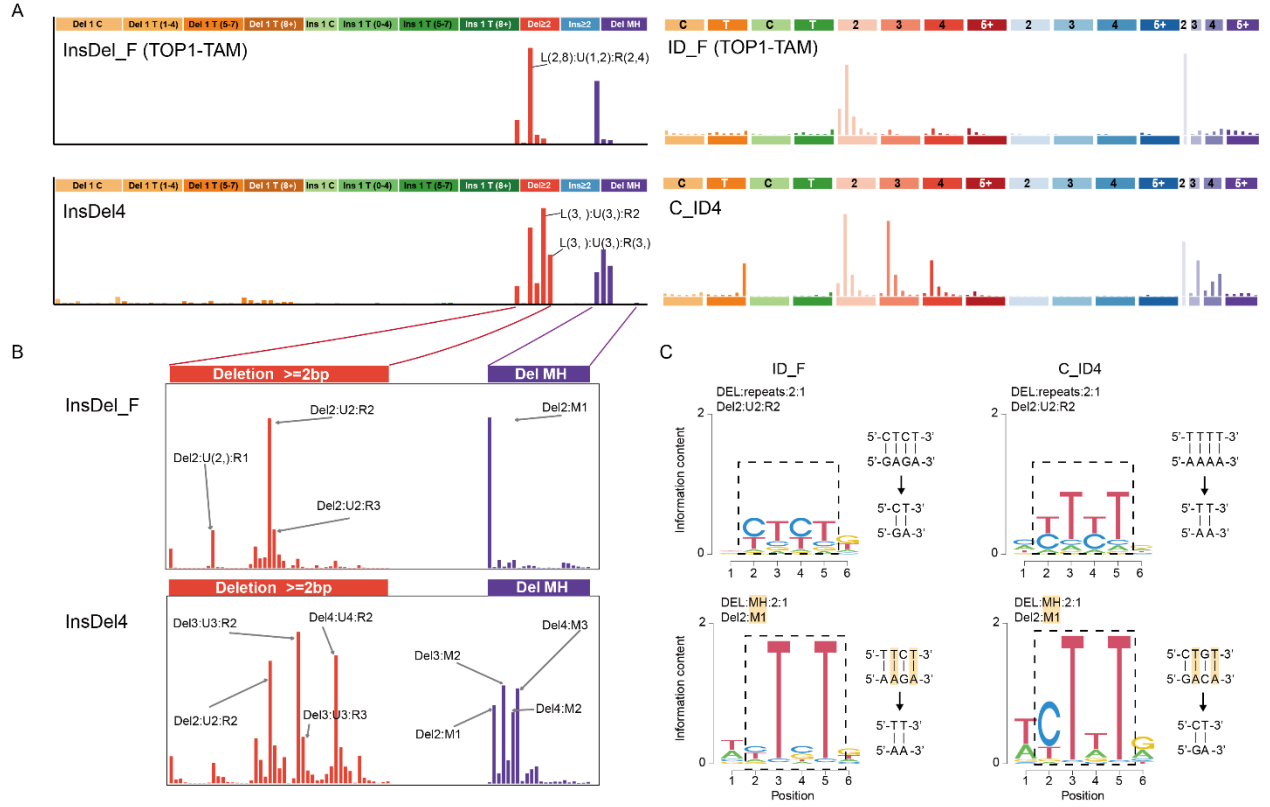


Figure 6: Novel signature caused by RNase H2 deficiency and topoisomerase-1-associated mutagenesis (TOP1-TAM)

(A) The mutational signature of RNase H2 deficiency (InsDel_F in the 89-type classification, ID_F in the 83-type classification), and a previously identified mutational signature (InsDel4, C_ID4).

(B) Deletions of 2 bases in the 476-type classification scheme in InsDel_F compared to InsDel4. InsDel4 has substantial proportions of deletions of 3 and 4 bases (Del3:U2:R2, Del4:U4:R2, Del3:M2, Del4:M2, Del4:M3) that are essentially absent in InsDel_F. Refer to panel B for additional explanations of the mutation labels.

(C) Logos of the sequence contexts of 2-base deletions in 5 samples with the highest InsDel_F activity (left) compared to 5 samples with the highest InsDel4 activity (right). The sequence contexts of the 2-base deletions in InsDel_F are on the whole different from the contexts of 2-base deletions in InsDel4. In the lower two sections (DEL:MH:2:1, 2-base deletions with 1 base of microhomology), the microhomologous bases (T:A) are shaded orange.